

Q Is it safe to use complementary therapies in pregnancy?

There are certain complementary therapies, or aspects of them, that are not safe for you or your baby during pregnancy. Your doctor is trained to support you in your decisions about your care and will guide you on what is appropriate and what's not, based on medical evidence. Avoid buying over-the-counter remedies, and consult a practitioner qualified in prenatal care before using any oil or herb.

» **Chiropractic:** useful for back and joint pain. Always look for a practitioner with a qualification in prenatal care.

» **Acupressure, acupuncture, and shiatsu:** can help with nausea and morning sickness in early pregnancy. Always go to a practitioner qualified in pregnancy care.

» **Bach flower remedies:** may help with stress or anxiety, if your doctor allows them. Clinical studies consider them to have a psychophysiological effect only—that is, feeling that you're taking control of an issue is the effective part of the treatment, rather than any active ingredient in the treatment itself.

» **Osteopathy:** useful for back and joint pain. Always choose an osteopath trained to treat pregnant women.

» **Homeopathy:** may help with nausea and vomiting, and fatigue. It is a matter of continuing medical debate as to how and why homeopathy works, so be sure that your doctor supports your decision before you move forward.

» **Hypnotherapy:** can help you manage labor pain. See a practitioner who can teach you self-help techniques to keep you relaxed and focused during your labor and birth.

» **Moxibustion:** reported to help turn a breech baby. Moxibustion is a form of traditional Chinese Medicine—use only if you receive your doctor's go-ahead.

» **Massage:** useful for stress, anxiety, back and joint pain. Find a masseuse trained in pregnancy care. You shouldn't lie on your back in the third trimester—instead lie on your side or try a foot, facial, or hand massage. Avoid essential oils.

Q What's my doctor feeling for when she presses on my belly?

Your doctor's pressing, stroking, and pushing movements across, above, and below your belly are known as “palpation.” He or she will do this primarily to determine the position of your baby from around 36 weeks onward.

By palpating your belly, your doctor will be able to tell when your baby has moved into the head-down position ready for birth, when your baby's head has “engaged” (moved downward into your pelvic area), whether your baby is curled over, head to chest (the ideal position for birth), or is more extended, and the position of the baby's

spine compared with your spine. As you lie on your back, your doctor will use both hands to smoothly and firmly feel all over your belly with the pads of his or her fingers. Try to stay relaxed throughout the procedure, since this makes the palpation more accurate. Let your doctor know if anything causes you any discomfort.



Fundal palpation This maneuver checks what part of your baby's body is at the top of your uterus. At 36 weeks, it's hoped to be your baby's bottom.



Lateral palpation Your doctor's hands move to the side of your abdomen so that he or she can check the position of your baby's spine.



Pelvic palpation This helps your doctor determine what part of your baby is in your pelvis. It's the most important maneuver in the palpation sequence.



Pawlick's grip This is a form of pelvic palpation and it tells your doctor whether your baby's head faces downward and is engaged in your pelvis.